SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Degarelix: hormone injections every 28 days

Advanced hormone-dependent prostate cancer · NCCP 00481 · version 3 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Schedule
The first dose is 240 mg as two 120 mg injections under the skin. The first 80 mg maintenance injection is 28 days later, then 80 mg every 28 days. The team sets the total duration.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Degarelix
GNRH RECEPTOR ANTAGONIST

- ☐ Hot flushes, sweating, fatigue and changes in sleep or mood
- ☐ Reduced sex drive, erection difficulties and reduced fertility
- ☐ Loss of bone strength, fracture risk, muscle loss and changes in weight
- ☐ Changes in glucose, cholesterol and cardiovascular risk
- ☐ Heart-rhythm changes; review other medicines that affect the QT interval
- ☐ Pain, redness, swelling or a lump at an injection site
- ☐ Allergic reactions and changes in liver blood tests

Discuss treatment aim, duration, benefits and uncertainty, alternatives, sexual health and fertility, hot flushes, fatigue, mood, bone and muscle effects, metabolic/cardiovascular risks, interactions, formulation-specific risks, tests and urgent contacts. Book the next injection before leaving. If an appointment is missed, contact the team promptly for a new date; do not arrange an extra loading dose yourself. Degarelix does not produce an initial testosterone surge, so anti-androgen treatment solely for flare prevention is not normally needed.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00481 v3.

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